

ATORVASTATIN TABLET

VIATOXX-M

Description

Hovid Atorvastatin Tablet 10 mg : White coloured film-coated oval shaped tablets, shallow convex faces, plain on both sides.
Hovid Atorvastatin Tablet 20 mg : White coloured film-coated oval shaped tablets, shallow convex with "HD" and break bar embossed on one face.

Composition

Each film-coated tablet contains :
Hovid Atorvastatin Tablet 10 mg : Atorvastatin Calcium equivalent to Atorvastatin 10 mg
Hovid Atorvastatin Tablet 20 mg : Atorvastatin Calcium equivalent to Atorvastatin 20 mg

Pharmacodynamics

Atorvastatin is a selective, competitive inhibitor of 3-Hydroxy-3-methylglutaryl coenzyme A (HMG-CoA) reductase. The inhibition of HMG-CoA reductase prevents conversion of HMG-CoA to mevalonate, the rate-limiting step in cholesterol biosynthesis. Inhibition of cholesterol synthesis in the liver leads to upregulation of low-density lipoprotein (LDL) receptors and an increase in catabolism of LDL cholesterol. It may also reduce the production of LDL as a result of inhibition of hepatic synthesis of very low-density lipoprotein (VLDL), the precursor of LDL.

HMG-CoA reductase inhibitors reduce LDL-cholesterol, VLDL-cholesterol, and to a lesser extent, plasma triglyceride concentrations, and slightly increase high-density lipoprotein (HDL) concentration.

Pharmacokinetics

Atorvastatin is rapidly absorbed after oral administration; maximum plasma concentration occurs within 1 to 2 hours. It has low absolute bioavailability of about 12% due to presystemic clearance in the gastrointestinal mucosa and / or first-pass metabolism in the liver, its primary sites of action.
Atorvastatin is 98% bound to plasma proteins. It is metabolized by ortho- and parahydroxylation and beta-oxidation to active metabolites. The drug and its metabolites are eliminated primarily in the bile. The mean plasma elimination half-life is approximately 14 hours.

Indications

Atorvastatin is indicated :
As an adjunct therapy to diet to reduce elevated total cholesterol, LDL-cholesterol, apolipoprotein B, triglycerides and increase HDL-cholesterol in patients with:
- Primary hypercholesterolemia (heterozygous familial and nonfamilial)
- Combined hyperlipidemia (Fredrickson Types IIa and IIb)
- Hypertriglyceridemia (Fredrickson Type IV)
- Primary dysbetalipoproteinemia (Fredrickson Type III)
As an adjunct therapy with other lipid lowering treatment (e.g. LDL apheresis) in homozygous familial hypercholesterolemia to reduce total cholesterol and LDL cholesterol.

Contraindications

- Active liver disease or unexplained persistent elevations of serum transaminases
- Hypersensitivity to any component of this drug
- Pregnancy and lactation

Precautions

- Liver Dysfunction: HMG-CoA reductase inhibitors have been associated with biochemical abnormalities of liver function. Therefore, liver function tests should be performed prior to and at 12 weeks following initiation of therapy or elevation in dose, and semiannually thereafter.
- Atorvastatin should be used with caution in patients with alcoholism, organ transplant with immunosuppressant therapy and/or patients who have a history of liver disease.
- Serious conditions such as hypotension, severe acute infection, severe metabolic, endocrine, or electrolyte disorder, uncontrolled seizures, major surgery, or trauma may increased risk of secondary renal failure if rhabdomyolysis occurs.

Pregnancy and Lactation

Atorvastatin is not recommended during pregnancy, in women who plan to become pregnant in the near future, or during lactation.

Main Side/Adverse Effects

Myalgia, myositis, rhabdomyolysis, constipation, diarrhea, heartburn, stomach pain, dizziness, headache, nausea, skin rash, insomnia.

Drug Interactions

- The risk of myopathy increased with concurrent administration of cyclosporine, fibric acid derivatives, niacin (nicotinic acid), erythromycin or azole antifungals.
- Co-administration with antacid reduces plasma concentrations of atorvastatin by approximately 35%.
- Concurrent use of atorvastatin with cholestyramine or colestipol may decrease the bioavailability of atorvastatin.
- Concurrent use with digoxin may cause an elevation in serum digoxin concentration.
- Concurrent use with anticoagulants, coumarin- or indandione derivative may increase bleeding or prothrombin time.
- Concurrent use with large amounts of grapefruit juice has been reported to significantly increase the serum concentrations and the area under the plasma concentration-time curve (AUC).
- Co-administration of atorvastatin with an oral contraceptive may increase AUC value for norethindrone and ethinyl estradiol by approximately 30% and 20% respectively.
- Plasma concentrations of atorvastatin increased with co-administration of erythromycin.
- Caution should be exercised when if an HMG-CoA reductase inhibitor is administered concomitantly with drugs that may decrease the levels or activity of endogenous steroid hormones, such as ketoconazole, spironolactone, and cimetidine.

Overdose

There is no specific treatment of atorvastatin overdose. In the event of an overdose, the patient should be treated symptomatically, and supportive measures instituted as required.

Dosage and administration

Usual Adult and adolescent dose :
Oral, the recommended initial dose of atorvastatin is 10 mg once daily which may be adjusted at intervals of 4 weeks. The dosage range is 10 to 60 mg once daily.

Usual pediatric dose :
Dosage has not been established.

The patient should be placed on a standard cholesterol-lowering diet and weight reduction programs/exercises before receiving atorvastatin and should continue the regimen during treatment with atorvastatin.

Note : The information given here is limited. For further information consult your doctor or pharmacist.

Storage : Store in a dry place at room temperature below 30°C. Protect from direct sunlight.
Presentation/Packing : Alu-alu blister of 3 x 10's.

Marketing authorization holder : HOVID Bhd., 121, Jalan Tunku Abdul Rahman, 30010 Ipoh, Malaysia.
Manufactured by : HOVID Bhd., Lot 56442, 7 1/2 Miles, Jalan Ipoh/Chemor, 31200 Chemor, Malaysia.

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